PEER REVIEW HISTORY

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ARTICLE DETAILS

TITLE (PROVISIONAL)	Psychometric properties of self-reported financial toxicity measures in cancer survivors: A systematic review
AUTHORS	Zhu, Zheng; Xing, Weijie; Wen, Huan; Sun, Yanling; So, WK; Lizarondo, Lucylynn; Peng, Jian; Hu, Yan

VERSION 1 – REVIEW

REVIEWER	Perrone, Francesco Istituto Nazionale Tumori IRCCS Fondazione PascaleCura dei , Clinical Trials Unit I must declare an intellectual conflict of interest because I'm the PI of the group that produced PROFFIT, the Italian instrument for measuring FT. This instrument was developed according to a protocol that was published (BMJ Open 2019;9:e031485); qualitative (Support Care Cancer 2021;29:3219-3233) and final analysis including internal validation (BMJ Open 2021;11:e049128) have been reported. Apparently, the qualitative paper (that was available in Pubmed as Epub since 2020 Oct 22) was excluded from the revision, maybe because it was not a "validation analysis". For transparency, I have reported this conflict of interest at the beginning of my revision. To the Editor: please feel free to not use this revision if you perceive that my conflict of interest is making my comments unbalanced.
REVIEW RETURNED	06-Dec-2021

GENERAL COMMENTS	I must declare an intellectual conflict of interest because I'm the PI of the group that produced PROFFIT, the Italian instrument for measuring FT. This instrument was developed according to a protocol that was published (BMJ Open 2019;9:e031485); qualitative (Support Care Cancer 2021;29:3219-3233) and final analysis including internal validation (BMJ Open 2021;11:e049128) have been reported. Apparently, the qualitative paper (that was available in Pubmed as Epub since 2020 Oct 22) was excluded from the revision, maybe because it was not a "validation analysis". That said, this is a systematic review of PRO instruments for measuring financial toxicity (FT) of cancer. The matter is important. The review is well written, but I have some concerns. My major concern is that including translations of available questionnaires does affect in my opinion the perception of the reported evidence that follows publication and development strategies rather than conceptual scientific advances. I mean, I really appreciate the great work the COST investigators are doing, but actually 8 out of 20 PROMS are language variants of the same COST instrument. And the conclusion (in the abstract) that COST is "the most suitable measures for use in research and clinical

	practice" might rather be written like "the only worldwide available measure…". Actually, my feeling is that there are very few instruments available for measuring FT, for sure not 20, maybe 4 to 5 worldwide. COST is the most diffused and should be considered as a benchmark for those who engage in the development of new instruments. And the reason why I began my revision citing PROFFIT, in addition to transparency, is because we called for attention on the need that instruments for measuring FT are developed within specific contexts, in terms of organization of health systems (public vs private to be clear, even accepting that many nuances actually exist) and in terms of level of richness of each single country or region. This had also been clearly described by Rotter J, et al. J Oncol Pract 15: e300-e307, 2019. The Authors did address this issue in discussion, but I would give much more emphasis also describing (and tabulating) the context where each instrument has been originally developed. Actually, I would suggest the Authors to reshape the way data are presented, avoiding to fall into an over-citation bias. I would also suggest to focus on instruments that have been planned for measuring FT. To be clear, item 28 of the EORTC C30 module may be useful to explore whether financial problems exist in a given setting, but I would not consider it as an "instrument to measure FT".

REVIEWER	El Adam, Shiraz BC Cancer Agency, Cancer control
REVIEW RETURNED	31-Dec-2021

GENERAL COMMENTS	In this systematic review, Zhu et al. summarize the psychometric properties of patient-reported outcome measures (PROMs) measuring financial toxicity (FT) among cancer survivors. This study addresses an important research question that could enable the standardization of FT measures. I have a few minor comments/suggestions: Abstract A strength of this review is that it was conducted according to the guidance of the Consensus-Based Standards for the Selection of Health Measurement Instruments (COSMIN) methodology and the Preferred Reporting Items for Systematic Reviews and Meta-Analyses (PRISMA). Comment 1: It is not clear why the PRISMA guidelines were not mentioned in the abstract, but some further details under the PRISMA 2020 for Abstracts Checklist are missing from the abstract. For instance, a summary of the eligibility criteria, information sources, and search period are missing from the methods section. Comment 2: It is helpful for readers to spell out abbreviated measures (COST, FIT, IOC-CS), especially when used for the first time in the paper. Comment 3: Since one of the stated strengths of this review is to "provide quantitative evidence for researchers and healthcare professionals", it will be helpful if the results section contains a summary of the measures included and quantifies how the "COST" showed better psychometric properties.

	Comment 4: The conclusion could benefit from a statement on the limitations of the summarized evidence. Methods Comment 5: The information sources under the search strategy section need some editing/clarification. This is because the search strategy subsection lists seven databases, without including PsycInfo, yet Appendix 1 and Figure 1 both include PsycINFO as a searched database. It is not clear whether this database was searched. If so, then the search strategy in the methods section should be updated to include it, as well as the number of databases searched in the abstract. Comment 6: The search period is missing from this manuscript, and appears inconsistent with the previously published protocol. In the published protocol, the abstract mentions a search "from inception to 15 November 2019", yet in the same protocol, the search strategy in the methods section lists a search from "inception to 1 March 2020". In Appendix 1, attached to this manuscript, only the dates of the conducted searches are listed, which seem to extend to 2021. It is important to clarify the dates spanning the literature search, not only for readers and researchers, but to allow the study to be easily repeated as well. Comment 7: In the inclusion and exclusion criteria section, it would be helpful to explain how the authors handled studies that reported for combined age groups of ≥18 years and <18. This is especially relevant since some of the selected articles included in Table 1 seem to have these age ranges combined. Comment 8: In the Quality Appraisal section, please reference the COSMIN Risk of Bias Checklist, and consider including it as an Appendix. This makes it easier for readers to follow the methodology and appraisal process. Comment 9: In the attached PRISMA checklist, there are references to Appendices 2 and 3, which aren't attached. I'm assuming those were updated to Tables 2 and 3, which would need to be reflected in the checklist accordingly. Results Comment 10: Tables 2 and 3 contain several abbreviated words and symbols that should be described in footnotes respectively. Comment 11: The readability of the measurement property assessment section could be improved if "+" and "-" symbols were replaced with their intended meaning, that is, sufficient or insufficient. Other minor comments: Comment 12: The abbreviated word "COSMIN" was first spelled-out in line 122, yet used earlier in line 110. Comment 13: References 19 and 45 are identical.

REVIEWER	de Alcantara Nogueira, Luciana Federal University of Paraná Clinics Hospital, Enfermagem
REVIEW RETURNED	31-Dec-2021

GENERAL COMMENTS	- Study 32 did not perform a criterion assessment. To review - The study affirms the importance of paying attention to the patient's economic issue - parents and health service, however, it does not reinforce that some studies performed this analysis. - in the "Quality appraisal" item, when a study did not perform any measurement, this disqualified. Explain - when this review was performed

REVIEWER	Chiesi, Francesca Univ Florence
REVIEW RETURNED	05-Jan-2022

GENERAL COMMENTS	Thank you for the opportunity to review the manuscript “Psychometric properties of self-reported financial toxicity measures in cancer survivors: A systematic review”. This is an extensive review of FT measures focused on their psychometric properties. Whereas I can appreciate the effort made in reviewing and assessing the literature on FT measures, I have a general concern about the methodology adopted in the evaluation of the selected papers. Although a protocol study was previously published to describe the methodology of the project, the criteria adopted for the evaluation are unclear. The reference to the COSMIN and GRADE protocols is not sufficient to understand the judgments provided. I try to better explain my point. p. 11, lines 162-169 “Two reviewers (WH & YS) assessed the methodological quality of the PROM of the included studies by using the COSMIN Risk of Bias Checklist. The checklist consisted of 10 domains (38 items), including PROM development, content validity, structural validity, internal consistency, cross-cultural validity, reliability, measurement error, criterion validity, hypothesis testing, and responsiveness. Each measurement property was rated as “very good”, “adequate”, “doubtful” or “inadequate quality”. The methodological quality of a single study is rated based on the worst score counts method. Any discrepancies were resolved by a third reviewer (ZZ)”. Whereas I can understand that is not possible to describe the full protocol, more explanations are needed to provide the reader some cues to interpret the data reported in Table 1-4. Thus, I suggest including short explanations for each evaluated property and providing some examples of “very good”, “adequate”, “doubtful” or “inadequate quality”. Moreover, why the methodological quality of a single study is rated based on "the worst score counts method”? This point deserves an explanation. p. 12, lines 186-187: “Each measurement property from each study was rated as sufficient (+), insufficient (−), or indeterminate (?).” Again, some examples might help the reader in understanding how each judgment was made. p. 12, lines 188-192 “If the ratings of one psychometric property per study were all sufficient or insufficient, the results were pooled, and the overall rating was rated as sufficient or insufficient. If the ratings were inconsistent, explanations of inconsistency were explored (e.g., different languages). If the explanation was reasonable, we provided ratings in subgroups. If the explanation was not reasonable, the overall rating of this measurement property was rated as inconsistent (±).” I'm sorry to say that this part is totally unclear to

	me. I cannot understand to which explanations the authors refer to, which are the subgroups, and what was considered reasonable and what was not. p. 12, lines 197-198 “We used a modified Grading of Recommendations Assessment, Development, and Evaluation (GRADE) system to assess the certainty of evidence. Each piece of evidence was graded for risk of bias, inconsistency, imprecision, and indirectness.” Again, more explanations and examples are needed, along with a bibliographic reference of this evaluation system, and the changes made (since a “modified” version was used). Finally, how does Table 4 differ from Table 2 and 3? How the “certainty of evidence of measurement properties” was found? Minor points: • p. 8, line 99: For the sake of completeness, I suggest to mention the ESAS-T (Ripamonti, C., Leporati, R., De Feo, G. et al. Italian version of the Edmonton Symptom Assessment System (ESAS)–Total Care (TC): development and psychometric validation in patients undergoing cancer treatment or follow-up. Support Care Cancer (2021). https://doi.org/10.1007/s00520-021-06594-y) that includes the assessment of FT. • p. 13, line 213-216 “All included studies were published from 1988 to 2021. Seven studies were conducted in the United States, four in the United Kingdom, two in Canada, and two in China (mainland and Hong Kong).25,39 One study was conducted in 12 counties in Europe and North America”. This report is incomplete because European, South America, and South Asia studies were not mentioned. • Tables need a legend for the acronyms/abbreviations used.

VERSION 1 – AUTHOR RESPONSE

Reviewer 1:
1. I must declare an intellectual conflict of interest because I’m the PI of the group that produced PROFFIT, the Italian instrument for measuring FT. This instrument was developed according to a protocol that was published (BMJ Open 2019;9:e031485); qualitative (Support Care Cancer 2021;29:3219-3233) and final analysis including internal validation (BMJ Open 2021;11:e049128) have been reported. Apparently, the qualitative paper (that was available in Pubmed as Epub since 2020 Oct 22) was excluded from the revision, maybe because it was not a “validation analysis”.
Answer: Thank you for your comments. We appreciate reviewer reported the conflict of interest and provide their recent papers regarding this topic. Due to our comprehensive search being conducted in May 2021, we are not able to find the paper “Cross-sectional study to develop and describe psychometric characteristics of a patient-reported instrument (PROFFIT) for measuring financial toxicity of cancer within a public healthcare system”. We believe this study is highly relevant and can be included in this SR. We updated our search (to February 2022) and results according to the reviewer’s suggestions. Changes could be found in line across methods and results section highlighted in red.

2. My major concern is that including translations of available questionnaires does affect in my opinion the perception of the reported evidence that follows publication and development strategies rather than conceptual scientific advances. I mean, I really appreciate the great work the COST investigators are doing, but actually 8 out of 20 PROMS are language variants of the same COST instrument. And the conclusion (in the abstract) that COST is “the most suitable measures for use in research and clinical practice” might rather be written like “the only worldwide available measure…”.
Answer: Eight language variants of COST were developed based on a rigid PROM localization process. These PROMs reported different levels of psychometric properties in different contexts.

Therefore, according to COSMIN guideline, language variants of the same PROM should be regarded as different PROMs when researchers analyze psychometric properties.
We also agree with reviewer that COST should be described as the most suitable worldwide measure rather than the best measure. Please see changes in line 64-69 and 410-417.

3. Actually, my feeling is that there are very few instruments available for measuring FT, for sure not 20, maybe 4 to 5 worldwide. COST is the most diffused and should be considered as a benchmark for those who engage in the development of new instruments. And the reason why I began my revision citing PROFFIT, in addition to transparency, is because we called for attention on the need that instruments for measuring FT are developed within specific contexts, in terms of organization of health systems (public vs private to be clear, even accepting that many nuances actually exist) and in terms of level of richness of each single country or region. This had also been clearly described by Rotter J, et al. J Oncol Pract 15: e300-e307, 2019. The Authors did address this issue in discussion, but I would give much more emphasis also describing (and tabulating) the context where each instrument has been originally developed.
Answer: Thank you for your suggestions. We agreed that contexts are crucial for analyzing FT in cancer survivors. This is also one of our goals to develop this systematic review to let readers know what PROMs could be used in your context. As suggested by reviewer, more information has been added in line 66-49 and 371-393.
Regarding the number of instruments of FT, FT might be the new word but definitely not a new concept. Regarding the terminology, “financial toxicity”, “financial burden” and “financial distress” are often used interchangeably in research and share a similar definition. There are already many measures that assess survivors’ subjective financial burden/distress. However, the majority of them did not report a validation process.

4. Actually, I would suggest the Authors to reshape the way data are presented, avoiding to fall into an over-citation bias. I would also suggest to focus on instruments that have been planned for measuring FT. To be clear, item 28 of the EORTC C30 module may be useful to explore whether financial problems exist in a given setting, but I would not consider it as an “instrument to measure FT”.
Answer: Regarding the item 28 of the EORTC C30 module, after discussing with our team members, we accepted reviewer’s suggestion and exclude this PROM in our SR. Please see changes across results section.

Reviewer 2:
1. Comment 1: It is not clear why the PRISMA guidelines were not mentioned in the abstract, but some further details under the PRISMA 2020 for Abstracts Checklist are missing from the abstract. For instance, a summary of the eligibility criteria, information sources, and search period are missing from the methods section.
Answer: Thank you for your suggestion. Eligibility criteria, information sources, and search period are added. We rephrased the abstract against PRISMA 2020 for Abstracts Checklist. Please see the changes in abstract.

2. Comment 2: It is helpful for readers to spell out abbreviated measures (COST, FIT, IOC-CS), especially when used for the first time in the paper.
Answer: As suggested by reviewer, the definitions of abbreviations were added across the manuscript and table 1-4.

3. Comment 3: Since one of the stated strengths of this review is to “provide quantitative evidence for researchers and healthcare professionals”, it will be helpful if the results section contains a summary of the measures included and quantifies how the “COST” showed better psychometric properties.
Answer: As suggested by reviewer, more numbers were added in abstract and results section. Please see line 57-62 and table 3.

4. Comment 4: The conclusion could benefit from a statement on the limitations of the summarized evidence.
Answer: The limitations of the evidence and implications for future evidence have been added in line 66-79.

5. Comment 5: The information sources under the search strategy section need some editing/clarification. This is because the search strategy subsection lists seven databases, without including PsycInfo, yet Appendix 1 and Figure 1 both include PsycINFO as a searched database. It is not clear whether this database was searched. If so, then the search strategy in the methods section should be updated to include it, as well as the number of databases searched in the abstract.
Answer: Sorry for the mistakes. We actually searched 8 databases including PubMed/MEDLINE, MEDLINE (Ovid), Embase (Ovid), CINAHL (EBSCO), PsycINFO (EBSCO), Web of Science, ProQuest Dissertations and Theses, and Cochrane Library (Wiley). We updated the number in manuscript. Please see changes in line 41-43 and 146-149.

6. Comment 6: The search period is missing from this manuscript, and appears inconsistent with the previously published protocol. In the published protocol, the abstract mentions a search "from inception to 15 November 2019", yet in the same protocol, the search strategy in the methods section lists a search from "inception to 1 March 2020". In Appendix 1, attached to this manuscript, only the dates of the conducted searches are listed, which seem to extend to 2021. It is important to clarify the dates spanning the literature search, not only for readers and researchers, but to allow the study to be easily repeated as well.
Answer: Sorry for this mistake. Many papers regarding FT measures have been published from 2019 to 2022. We want to update our search to include more studies as possible. Therefore, in this version, we research all databases and updated our results until February 2022. Two additional papers published in 2021 and 2022 were added. Explanations for the inconsistency between protocol and this manuscript have been added in line 149-152.

7. Comment 7: In the inclusion and exclusion criteria section, it would be helpful to explain how the authors handled studies that reported for combined age groups of ≥18 years and <18. This is especially relevant since some of the selected articles included in Table 1 seem to have these age ranges combined.
Answer: Thank you for this suggestion. If the studies reported results in a population combined with both ≥18 and <18 years old cancer survivors and the majority of survivors were not < 18 years old, the studies were also considered. We added this information in line 164-167.

8. Comment 8: In the Quality Appraisal section, please reference the COSMIN Risk of Bias Checklist, and consider including it as an Appendix. This makes it easier for readers to follow the methodology and appraisal process.
Answer: As suggested by reviewer, the citation has been added in line 185. The COSMIN Risk of Bias Checklist was submitted as Appendix II.

9. Comment 9: In the attached PRISMA checklist, there are references to Appendices 2 and 3, which aren't attached. I'm assuming those were updated to Tables 2 and 3, which would need to be reflected in the checklist accordingly.
Answer: Sorry for the mistakes. It should be Table 2 and 3. Please see the changes in PRISMA checklist.

10. Comment 10: Tables 2 and 3 contain several abbreviated words and symbols that should be described in footnotes respectively.
Answer: Sorry for the mistakes. Abbreviations were added after Table 1-4.

11. Comment 11: The readability of the measurement property assessment section could be improved if "+" and "-" symbols were replaced with their intended meaning, that is, sufficient or insufficient.
Answer: Thank you for your suggestions. We fully understand your concerns. After adding effect sizes of each property, there might be too many words in Table 3 if we change "+" and "-" symbols to "sufficient" or "insufficient".

12. Comment 12: The abbreviated word "COSMIN" was first spelled-out in line 122, yet used earlier in line 110.
Answer: This problem has been fixed. Please see changes in line 137-138.

13. Comment 13: References 19 and 45 are identical.
Answer: Thank you so much. Reference 45 has been replaced.

Reviewer 3:
1. Study 32 did not perform a criterion assessment.
Answer: Thank you for your comments. Yes, study 32 (de Alcantara Nogueira et al., 2020) did not report criterion validity. This PROM only reported structural validity, internal consistency, and cross-cultural validity.

2. The study affirms the importance of paying attention to the patient's economic issue - parents and health service, however, it does not reinforce that some studies performed this analysis.
Answer: Salman and colleagues conducted a systematic review and found 8 PROMs and 2 caregiver-reported measures for assessing financial burden in adolescents and young adults. However, this review focused only on PROMs assessing FT in adolescents and young adults with cancer. The psychological properties of FT measures in adult cancer survivors are still unknown. As suggested by reviewer, this information was added in line 126-130.

3. In the "Quality appraisal" item, when a study did not perform any measurement, this disqualified. Explain
Answer: In our previous version, we consider “PROM Development” as one type of psychological property. Many reviewers thought this was not suitable. After discussing with our team members, we decide to delete two PROMs (EORTC QLQ-C30 Financial impact domain and CanCORS insurance coverage and income domain) from three papers (Aaronson et al., 1988, Aaronson et al., 1993, and Malin et al., 2006) in this version. Please see changes across the results section.

4. when this review was performed
Answer: This review was conducted in May 2021. In this version, we updated our search to March 2022 to include more recent published PROMs.

Reviewer 4:
1. Thank you for the opportunity to review the manuscript “Psychometric properties of self-reported financial toxicity measures in cancer survivors: A systematic review”. This is an extensive review of FT measures focused on their psychometric properties. Whereas I can appreciate the effort made in reviewing and assessing the literature on FT measures, I have a general concern about the methodology adopted in the evaluation of the selected papers. Although a protocol study was previously published to describe the methodology of the project, the criteria adopted for the evaluation are unclear. The reference to the COSMIN and GRADE protocols is not sufficient to understand the judgments provided. I try to better explain my point.
Answer: Thank you for your suggestions. We understand your concerns about the manuscript. Please see our comments below according to your comments.

2. p. 11, lines 162-169 “Two reviewers (WH & YS) assessed the methodological quality of the PROM of the included studies by using the COSMIN Risk of Bias Checklist. The checklist consisted of 10 domains (38 items), including PROM development, content validity, structural validity, internal consistency, cross-cultural validity, reliability, measurement error, criterion validity, hypothesis testing, and responsiveness. Each measurement property was rated as “very good”, “adequate”, “doubtful” or “inadequate quality”. The methodological quality of a single study is rated based on the worst score counts method. Any discrepancies were resolved by a third reviewer (ZZ)”. Whereas I can understand that is not possible to describe the full protocol, more explanations are needed to provide the reader some cues to interpret the data reported in Table 1-4. Thus, I suggest including short explanations for each evaluated property and providing some examples of “very good”, “adequate”, “doubtful” or “inadequate quality”. Moreover, why the methodological quality of a single study is rated based on "the worst score counts method”? This point deserves an explanation.
Answer: The COSMIN Risk of Bias checklist is very complex and has 116 items. (https://cosmin.nl/wp-content/uploads/COSMIN_risk-of-bias-checklist_dec-2017.pdf). Not all items in the checklist have the choice of “very good”, “adequate”, “doubtful” or “inadequate quality”. The criteria are totally different for 116 items.
For example: In 1a. PROM design domain (item 1 & 2)
very good adequate doubtful inadequate quality NA
1. Is a clear description provided of the construct to be measured? Construct clearly described Construct not clearly

described
2. Is the origin of the construct clear: was a theory, conceptual framework or disease model used or clear rationale provided to define the construct to be measured? Origin of the
construct clear Origin of the construct
not clear
To help better understand the content of the COSMIN checklist, we added Appendix II to include the whole checklist as a supplementary. We also added more explanations for reasons of using "the worst score counts method". Please see line 190-195.

3. p. 12, lines 186-187: "Each measurement property from each study was rated as sufficient (+), insufficient (−), or indeterminate (?)." Again, some examples might help the reader in understanding how each judgment was made.
Answer: Ten domains had different standards for sufficient (+), insufficient (−), or indeterminate (?). As suggested, we added the criteria for measurement property rating in Appendix II (Table 2 Criteria for good measurement properties). Please see changes in line 214-224 and Appendix II.

4. p. 12, lines 188-192 "If the ratings of one psychometric property per study were all sufficient or insufficient, the results were pooled, and the overall rating was rated as sufficient or insufficient. If the ratings were inconsistent, explanations of inconsistency were explored (e.g., different languages). If the explanation was reasonable, we provided ratings in subgroups. If the explanation was not reasonable, the overall rating of this measurement property was rated as inconsistent (±)." I'm sorry to say that this part is totally unclear to me. I cannot understand to which explanations the authors refer to, which are the subgroups, and what was considered reasonable and what was not.
Answer: Sorry for the confusion. We added some examples to help readers better understand the methods. In our case, we did not pool our results together because we believed different language, social, economic, and cultural contexts may contribute to the inconsistency of psychometric property. Therefore, we provided the results of psychometric property in each language version. Now, we rephrased this paragraph as suggested by reviewer. Please see changes in line 214-224.

5. p. 12, lines 197-198 "We used a modified Grading of Recommendations Assessment, Development, and Evaluation (GRADE) system to assess the certainty of evidence. Each piece of evidence was graded for risk of bias, inconsistency, imprecision, and indirectness." Again, more explanations and examples are needed, along with a bibliographic reference of this evaluation system, and the changes made (since a "modified" version was used).
Answer: We added an instruction of downgrading for risk of bias, inconsistency, imprecision, and indirectness in Appendix II (eTable 3). We believe the instruction is brief and understandable.

6. Finally, how does Table 4 differ from Table 2 and 3? How the "certainty of evidence of measurement properties" was found?
Answer: Table 2 shows the results of the quality of research methodology.
Table 3 shows the rating of the pooled or summarized results against the criteria for good measurement properties. The ratings of table 3 were more focused on the effect size of psychometric properties.
Table 4 shows the certainty of evidence of measurement property. The quality of the evidence refers to the confidence that the pooled or summarized result is trustworthy. The ratings of Table 4 were the final results of recommendations considering both effect sizes and research methodology.

7. p. 8, line 99: For the sake of completeness, I suggest to mention the ESAS-T (Ripamonti, C., Leporati, R., De Feo, G. et al. Italian version of the Edmonton Symptom Assessment System (ESAS)–Total Care (TC): development and psychometric validation in patients undergoing cancer treatment or follow-up. Support Care Cancer (2021). https://doi.org/10.1007/s00520-021-06594-y) that includes the assessment of FT.
Answer: Thank you for your suggestion. Although this study did not meet our inclusion criteria, we would like to cite this paper in our Introduction. Please see line 115 and citation 19.

8. p. 13, line 213-216 "All included studies were published from 1988 to 2021. Seven studies were conducted in the United States, four in the United Kingdom, two in Canada, and two in China (mainland and Hong Kong).25,39 One study was conducted in 12 counties in Europe and North America". This report is incomplete because European, South America, and South Asia studies were

not mentioned.
Answer: As suggested by reviewer, we added a sentence regarding studies conducted in Brazil, India, Iran, and Italy. Please see line 250-254.

9. Tables need a legend for the acronyms/abbreviations used.
Answer: As suggested, all abbreviations were added in Table 1-4.

VERSION 2 – REVIEW

REVIEWER	Perrone, Francesco Istituto Nazionale Tumori IRCCS Fondazione PascaleCura dei , Clinical Trials Unit I am involved in the development of one of the intrument described in this paper (PROFFIT)
REVIEW RETURNED	01-Apr-2022

GENERAL COMMENTS	The Authors answered my criticisms.

REVIEWER	Chiesi, Francesca Univ Florence
REVIEW RETURNED	01-Apr-2022

GENERAL COMMENTS	I found my comments adequately addressed and the review is overall improved and updated.